By Team Orange
Old Dominion
University
CS 410
2/10/2026

Patient Path

“From Fax to Fast: Modernizing Medical Record Exchange”

TABLE OF CONTENTS

Who We Are 3
Primary Offering 4
The Problem 6
The Solution 12
Risks 21
Market Assessment 22
Conclusions 23
References/Citations 24
Glossary 25

Who we are

Kaitlyn Molaison
Project Manager

Kaitlyn is the project manager of PatientPath, contributing to design, development, and documentation while helping guide the team's overall vision and progress.

James Woodson
Software Developer

James is a PatienPath team member who contributes to software design implementation as well as other backend tasks.

Darnell Barnes
Full Stack Developer

Darnell is a PatientPath team member who contributes to the frontend user interface and backend system functionality.

Who we are

Grace Wright
Front End Developer

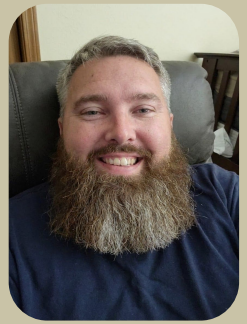
Grace is a PatientPath team member responsible for the front end development. She will contribute to the front end design of the website, and ensure that it is working properly for user's.

Christopher DeHaven
Software Developer

Christopher is a developer for PatientPath who contributes to software design implementation and improving functionality and efficiency within the system.

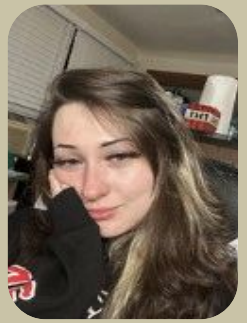
Stephen Garrison
Software Developer

Stephen Garrison is a team member on PatientPath who contributes to system design, technical writing, and building clear project communication through the team website.



Emily Nowak
Documentation Specialist

Emily is a PatientPath team member responsible for documentation and creating process flow diagrams to clearly communicate system design and project goals.



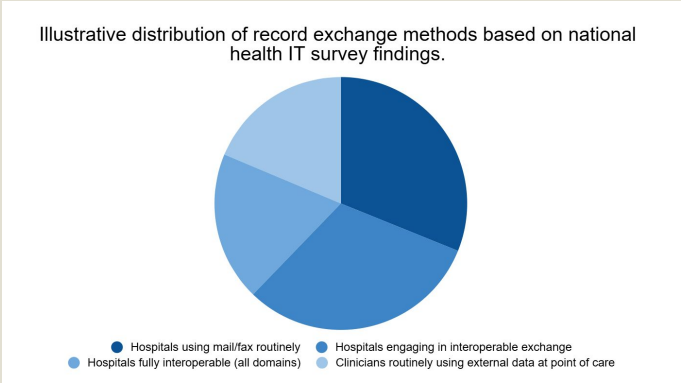
Elevator Pitch

PatientPath is a secure web-based platform that replaces outdated fax systems in healthcare with instant digital record sharing. It allows providers to access and share patient records in real time while integrating with existing EHR (electronic health record) systems. By reducing delays, errors, and administrative work, PatientPath helps deliver faster, safer, and more coordinated care, so patients’ medical stories follow them wherever they go.

The Problem

Current medical transfer methods are inefficient and slow.

- 65% of all transfers rely on fax machines despite the availability of digital alternatives
- Slow, unreliable, and prone to errors
  - Issues include lost pages, unreadable documents, and delayed delivery
- Direct impact on patient care
  - Leads to postponed diagnosis and repeated tests
- Disproportionate Harm
  - Especially impactful in rural and underfunded facilities



Office of the National Coordinator for Health Information Technology. (2023). *Interoperable exchange of patient health information among U.S. hospitals: 2023 data brief.* U.S. Department of Health & Human Services. https://www.healthit.gov/data/data-briefs/interoperable-exchange-patient-health-information-among-us-hospitals-2023

Current State of Healthcare IT

Although some healthcare practices have implemented EHR systems, *interoperability still remains a major challenge*.
Key issues include:

- Many EHR systems do not communicate across different organizations.
- Healthcare IT systems remain siloed.
- Smaller and rural communities often lack modern infrastructure.
- Regulatory requirements (HIPAA) increase complexity.

As a result, when systems cannot exchange data directly, providers often default to manual or outdated communication methods.

Source: Office of the National Coordinator for Health Information Technology (2023). Interoperable exchange of patient health information among U.S. hospitals.

Why This Matters

Healthcare is very time sensitive. Delays in information transfer can lead to:

- Postponed specialist appointments
- Repeated diagnostic testing
- Incomplete patient histories
- Increased workload for administrative staff
- All around frustration for patients and providers

In an industry where minutes can affect outcomes, inefficiencies in information exchange have real clinical and financial consequences.

Problem Characteristics

Patient Privacy Risk

Outdated security protocols

Confidential data vulnerability

Disconnected Systems

Limited interoperability

Inefficient Transfer Process

Record delays

Document loss

Illegible fax transmissions

Problem Characteristics

Scalability Barriers

Limited growth adaptability

Rigid architecture

Poor Usability

Cluttered interfaces

Poor navigation

Lack of user roles

Connectivity Constraints

Online-only architecture

Downtime vulnerability

Who is Affected?

- Patients
- Providers
- Admin Staff
- Rural Facilities
- Families/Caregivers

Current Flow Process

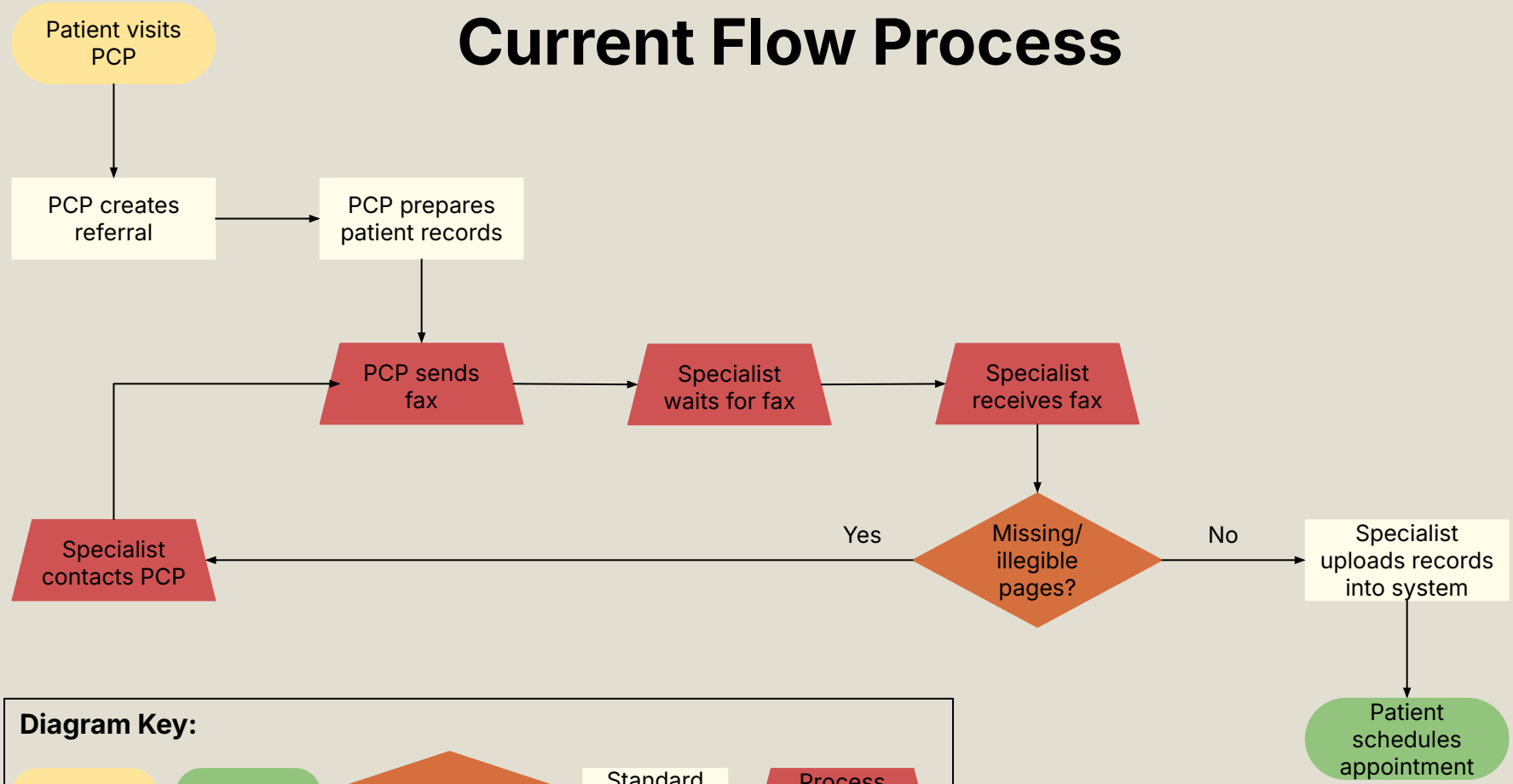


Diagram Key:

Start | Stop | Decision | Standard Step | Process Bottleneck

The Solution

PatientPath is a secure, centralized digital platform designed to replace outdated fax-based communication in healthcare.

Instead of faxing documents, PatientPath enables providers to instantly upload, access, and share medical records through a secure web-based system.

The platform supports hospitals, clinics, private practices, healthcare networks, and their patients.

Unlike traditional EHR systems, PatientPath focuses on secure interoperability and cross-organization communication. It acts as a **secure bridge between systems**, reducing reliance on fax while improving cybersecurity, compliance, and usability.

Solution Characteristics

Security & Compliance

- End-to-end encryption (data in transit + at rest)
- HIPAA-aligned architecture
- Role-Based Access Control (organization-level permissions)
- Multi-factor authentication
- Full audit trail logging

Solution Characteristics

Interoperability & Architecture

- API-first REST architecture
- FHIR & HL7 compatibility
- EHR-agnostic integration (Epic, Oracle, etc.)
- Plugin-based deployment model

Solution Characteristics

Performance & Scalability

- Cloud-hosted (AWS/Azure/GCP compatible)
- 99.9%+ uptime target
- Horizontal scaling architecture
- Offline sync resilience

Solution Process Flow

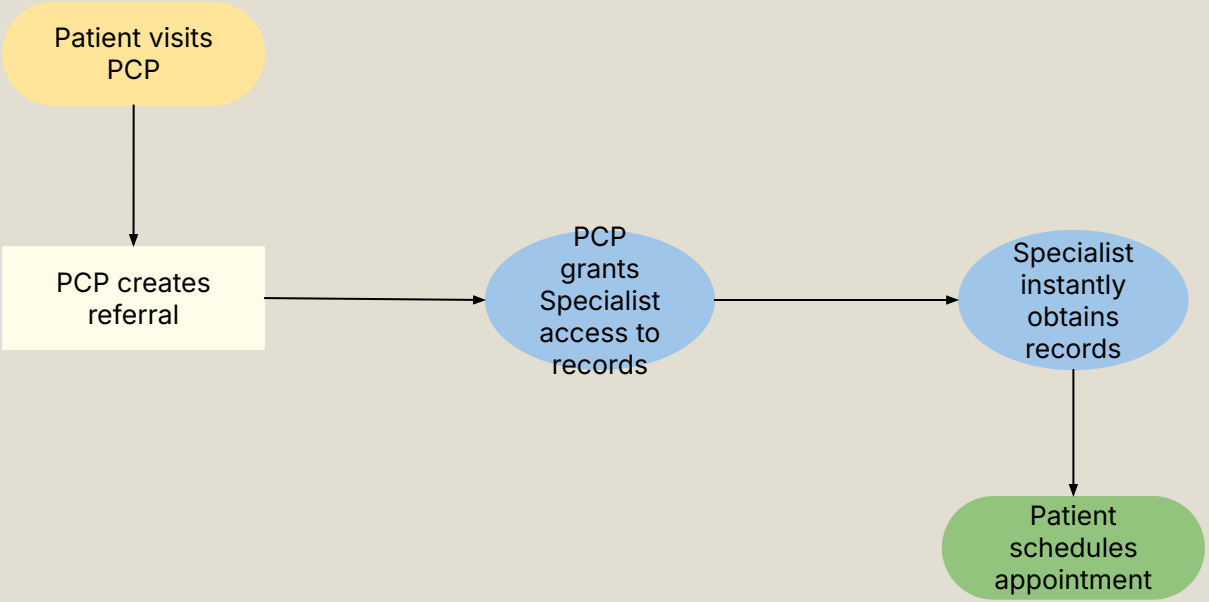


Diagram Key:

Problem–Solution Mapping

	Identified Problem	System Feature	Expected Impact
1	Reliance on fax-based record transfers	Secure real-time digital document exchange	Reduced delays & elimination of document loss
2	Limited interoperability between EHR systems	API-first architecture with FHIR/HL7 compatibility	Standardized cross-organization data exchange
3	Manual referral tracking & follow-up processes	Automated referral workflow with status monitoring	Improved efficiency & faster referral completion
4	Communication methods withsecurity vulnerabilities	End-to-end encryption with RBAC and MFA	Enhanced HIPAA-aligned compliance & access control
5	Connectivity instability in rural / low-resource areas	Offline caching with automatic synchronization	Sustained usability in low-bandwidth environments

Major Functional Components Architecture

Presentation Layer

- Provider Dashboard
- Patient Portal
- Admin Console
- Login & MFA Screens
- Notification Interface

USERS

Patients

Providers

Administrators

Application Layer

- *API Gateway / Web Server*
- Identity & Access Management
- Patient Record Management
- Secure Record Exchange Service
- FHIR / HL7 Integration Service

Data Layer

- PostgreSQL Database (user & organization)
- Encrypted Document Storage (Medical Records)
- Audit Logs
- Consent Records
- FHIR Normalized Data

Patient Path Will

What Patient Path Will Do

- Enable secure digital transfer of medical records between providers
- Provide a centralized hub for accessing and managing patient documents
- Offer real-time tracking of record requests and transfer status
- Support digital patient consent and authorization workflows
- Reduce errors with automated validation and completeness checks
- Maintain HIPAA-compliant audit trails and access controls

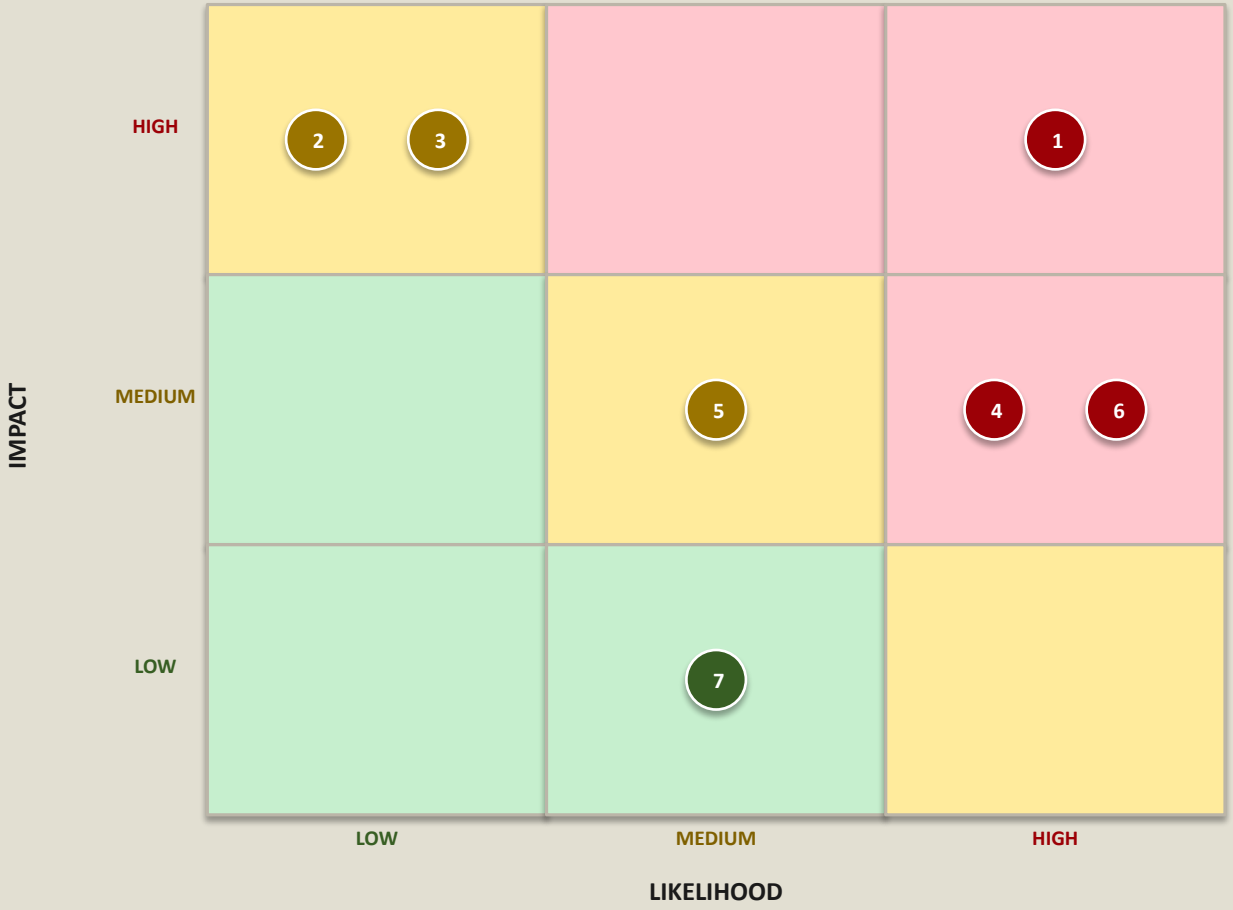
Patient Path Will Not

Provide medical diagnoses or clinical recommendations - The platform does not assist in medical decision making. It’s role is limited to securely transferring and accessing information.

Handle hospital management functions - Scheduling, billing, and internal hospital operations are already handled by specialized systems.

Function as an emergency communication system - The platform supports data exchange, not urgent/emergency situations.

Risk Assessment Matrix



Risk Legend

HIGH — Technical
MED — Customer
LOW — Legal/Sec

1. EHR Integration Complexity
   Technical • Control
2. Cybersecurity / Data Breach
   Legal & Sec • Control
3. HIPAA Non-Compliance
   Legal & Sec • Avoid
4. Provider Adoption Resistance
   Customer • Control
5. System Downtime / Cloud Failure
   Technical • Transfer
6. Scope Creep
   Technical • Avoid
7. Patient Trust / Privacy Concerns
   Customer • Monitor

Risk Mitigation Strategies

Organized by risk category | Approach types: Control · Avoid · Transfer · Monitor

Technical Risks

1 EHR Integration Complexity

Control

Phased FHIR/HL7 rollout with sandbox testing before production deployment

5 System Downtime / Cloud Failure

Transfer

SLA with cloud provider (AWS/Azure) requiring 99.9%+ uptime; failover redundancy

6 Scope Creep

Avoid

Strict stage-gate process with weekly milestone reviews and frozen requirements

Customer Risks

4 Provider Adoption Resistance

Control

Structured onboarding program, intuitive UI design, and workflow alignment workshops

7 Patient Trust / Privacy Concerns

Monitor

Transparent consent flows, clear privacy policy, and regular patient communication

Legal & Security Risks

2 Cybersecurity / Data Breach

Control

End-to-end encryption, MFA, annual penetration testing & incident response plan

3 HIPAA Non-Compliance

Avoid

Legal review at each dev phase; automated compliance checks built into CI/CD pipeline

Market Assessment

Target Segments

- Hospitals and Healthcare networks
- Independent private practices
- Specialty Clinics

Competitive Landscape

- Traditional fax transfers
- Electronic Health Record systems (Epic, Oracle Healthcare, and eClinicWorks)

Competition Comparison Matrix

Capability	PatientPath	Traditional Fax	Kno2 (Digital Fax)	Redox (Healthcare APIs)	CommonWell HIE
Real-time record transfer	✓	✗	Limited	✓	✓
Cross-organization data exchange	✓	✓ (manual)	✓	✓	✓
EHR interoperability (FHIR/HL7)	✓	✗	Limited	✓	✓
Automated referral workflow	✓	✗	Limited	✗	✗
Secure audit logging	✓	✗	✓	✓	✓
Offline support for rural clinics	✓	✓	✗	✗	✗
Ease of deployment	✓	✓	✓	Limited	Limited

User Roles

Primary Users of the PatientPath System

Healthcare Providers

Doctors, Specialists, PCPs

- Upload and share patient medical records
- Access records from other organizations in real-time
- Initiate and track referrals
- Grant and manage access permissions

Administrative Staff

Record & Workflow Management

- Manage patient documentation and record requests
- Monitor referral workflows and status updates
- Ensure completeness of transferred records
- Handle communication between organizations

Patients

Read-Only Access & Consent

- Provide consent for record sharing
- View transfer status of their medical records
- Access personal health documents (read-only)
- Maintain control over who can access their data

System Administrators

Security, Compliance & Configuration

- Manage user accounts and roles (RBAC)
- Monitor system security and audit logs
- Configure integrations with EHR systems
- Ensure compliance with HIPAA and security policies

User Stories — Healthcare Providers & Admin Staff

Healthcare Provider — US-1

Secure Record Upload

As a healthcare provider, I want to upload patient records securely so that I can share them with other providers in real time.

Acceptance Criteria:

- File is AES-256 encrypted on upload
- Only authorized users can access records
- Upload completes within the defined time threshold
- System validates file format and completeness

→ **Trace:** *Secure Record Upload → Provider Dashboard (upload form) → ValidateAndEncrypt module → Encrypted Doc Storage + PostgreSQL → Dashboard (confirmation)*

Healthcare Provider — US-2

Referral Initiation

As a healthcare provider, I want to initiate a referral and grant record access in a single workflow so that I can eliminate manual faxing.

Acceptance Criteria:

- Referral created with one form submission
- Access granted to receiving organization immediately
- Provider receives confirmation notification
- Incomplete packages are blocked with specific error messages

→ **Trace:** *Referral Tracking + Real-Time Sharing → Provider Dashboard (referral form) → CreateReferral module → PostgreSQL + IAM → Specialist Dashboard (notification)*

Administrative Staff — US-3

Referral Status Monitoring

As an administrative staff member, I want to monitor the status of all active referrals so that I can eliminate manual phone follow-ups.

Acceptance Criteria:

- Dashboard displays real-time status for all referrals
- Status updates within 30 seconds of a change
- Staff can filter by provider, date, or status
- Completed referrals are archived and searchable

→ **Trace:** *Referral Tracking → Admin Console (dashboard) → TrackReferralStatus module → PostgreSQL → Admin Console (real-time update + 48hr alert)*

Administrative Staff — US-4

Incomplete Record Flag

As an administrative staff member, I want the system to flag incomplete record packages before transmission so that referrals are not rejected.

Acceptance Criteria:

- System validates required fields before allowing transmission
- Incomplete packages are blocked with a specific error message
- Staff can resolve issues and resubmit without starting over
- Validation runs automatically on every save action

User Stories — Patients & System Administrators

Patient **US-9**

Digital Consent

As a patient, I want to provide digital consent for record sharing so that I maintain control over who accesses my health data.

Acceptance Criteria:

- Consent workflow accessible from patient portal
- Consent is timestamped, logged, and auditable
- Patient can revoke access at any time
- Provider is notified immediately upon revocation

→ **Trace:** *Patient Consent Mgmt → Patient Portal (consent form) → ManageConsent module → Consent Records store → Portal (confirmation) + Provider Dashboard (access update)*

Patient **US-10**

Transfer Status View

As a patient, I want to view the real-time transfer status of my records so that I am informed about my referral progress.

Acceptance Criteria:

- Portal shows current status in plain, non-technical language
- Status updates reflect changes within 60 seconds
- Historical transfers are visible and date-stamped
- Patient receives notification when records are accessed

System Administrator **US-11**

Immutable Audit Trail

As a system administrator, I want all data access events logged in an immutable audit trail so that HIPAA compliance requirements are met.

Acceptance Criteria:

- Every access, transfer, and login event is logged
- Logs include timestamp, user ID, and action type
- Logs cannot be edited or deleted by any user
- Audit reports are exportable in CSV and PDF format

→ **Trace:** *Audit Logging → (triggered by all system actions) → LogAccessEvent module → Audit Logs DB (append-only) → Admin Console (report export)*

System Administrator **US-12**

RBAC Management

As a system administrator, I want to manage user roles through RBAC so that access is restricted to authorized personnel only.

Acceptance Criteria:

- Roles can be assigned, modified, and revoked in real time
- Permission changes take effect immediately upon save
- Unauthorized access attempts trigger an automated alert
- Role history is logged for each user account

User Stories — Offline Resilience & Edge Cases

Healthcare Provider (Offline) — US-5

Offline Record Access

As a healthcare provider, I want to access cached records during an outage so that rural clinic downtime does not interrupt patient care.

Acceptance Criteria:

- Records accessed during a session are cached locally
- Cached data remains accessible during full network outage
- Queued writes sync automatically within 60 sec of reconnection
- No data loss occurs during sync; server state resolves conflicts

→ **Trace:** *Offline Sync → Provider Dashboard (offline indicator) → CacheRecords + SyncQueue modules → IndexedDB (local) sync to PostgreSQL → Dashboard (sync confirmation)*

Healthcare Provider (Edge Case) — US-6

Invalid Transfer Rejection

As a healthcare provider, I want the system to reject invalid or incomplete record transfers so that corrupt or partial data never reaches the receiving provider.

Acceptance Criteria:

- System blocks transmission of files failing format validation
- Error message specifies which fields or files are missing
- No partial data is transmitted under any condition
- Provider must resolve all errors before resubmission

System Administrator (Security) — US-7

Unauthorized Access Alert

As a system administrator, I want unauthorized access attempts to trigger an automatic alert so that potential breaches are detected promptly.

Acceptance Criteria:

- Three failed login attempts lock the account automatically
- Admin receives an alert within 30 seconds of lockout
- All attempts are logged with IP and timestamp
- Account unlock requires MFA verification by the user

Administrative Staff (Edge Case) — US-8

Stale Referral Alert

As an administrative staff member, I want the system to alert me when a referral is pending without action for 48 hours so that no patient falls through the cracks.

Acceptance Criteria:

- System monitors all active referrals for inactivity
- Alert sent to assigned admin after 48 hours of no update
- Admin can mark as reviewed to reset the timer
- Unresolved items are escalated to supervisor after 72 hours

Feature Table (1 of 3)

★ = Novel / Differentiating Feature

Feature	Description	User Roles	Priority	Complexity	Maps To	★
Core Functionality						
Secure Record Upload	Files are AES-256 encrypted and validated for format and completeness before upload. Invalid files blocked at API Gateway; approved records route to Encrypted Document Storage. Upload completes within 5 seconds.	Provider, Admin	**Core**	**Medium**	US-1, US-6	
Real-Time Record Sharing	IAM issues a scoped, time-limited access token to the receiving org upon referral creation. Provider dashboard updates via WebSocket; token scope limited to referenced records only. No manual steps required.	Provider	**Core**	**High**	US-1, US-2	★
Referral Tracking System	Single-form referral initiation triggers pre-send completeness validation. Status transitions (Pending → In Review → Accepted → Complete) stored in PostgreSQL and pushed to dashboards in real time. Stale referrals (48hr) trigger admin alerts.	Provider, Admin	**Core**	**High**	US-2, US-3, US-4, US-8	★
Patient Consent Management	Structured consent form in Patient Portal; signed consents timestamped and stored in Consent Records store. Revocations propagate to all active IAM sessions immediately, revoking access tokens in real time.	Patient, Provider	**Core**	**Medium**	US-9	★
Centralized Record Access	Unified cross-organization view of patient records via the API Gateway. Records normalized to FHIR R4 format for consistent display regardless of source EHR system.	Provider, Patient	**Core**	**Medium**	US-1, US-10	

Feature Table (2 of 3)

★ = Novel / Differentiating Feature

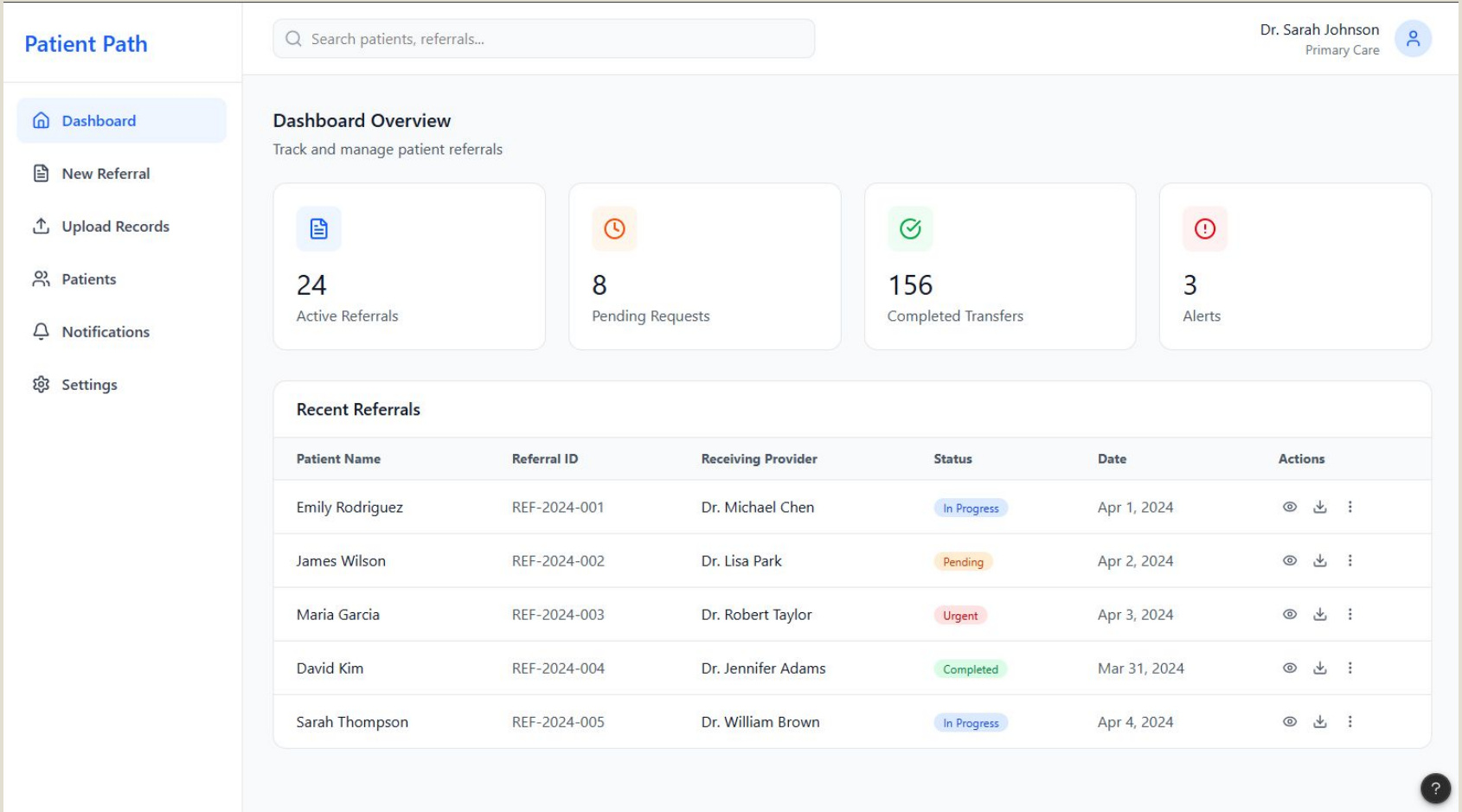
Feature	Description	User Roles	Priority	Complexity	Maps To	★
Security & Compliance						
End-to-End Encryption	AES-256 at rest; TLS 1.3 in transit. API Gateway terminates TLS before routing to internal services. Keys rotate every 90 days via cloud KMS (AWS KMS / Azure Key Vault).	All	**Core**	**High**	US-1, US-11	
Role-Based Access Control	Four roles (Provider, Admin, Patient, SysAdmin) enforced per API request by IAM. Permission changes propagate immediately; unauthorized attempts logged and trigger real-time alerts.	Admin	**Core**	**High**	US-7, US-12	
Multi-Factor Authentication	TOTP two-step verification required on all accounts. Three failed attempts trigger automatic lockout and admin alert within 30 seconds. Unlock requires MFA re-verification.	All	**Core**	**Low**	US-7	
Audit Logging	Every access, transfer, and login event written to an immutable log with timestamp, user ID, IP, and action type. Non-editable, exportable as CSV/PDF, retained per HIPAA.	Admin	**Core**	**Medium**	US-11	
Interoperability						
FHIR/HL7 Integration	Normalizes EHR data to FHIR R4 via a configurable mapping layer. Outbound data serialized to FHIR or HL7 v2.x. Supports RESTful FHIR endpoints and HL7 message queuing.	System Admin	**Core**	**High**	US-1, US-2	★
EHR Integration Layer	Plugin-based adapters connect to Epic, Oracle, eClinicalWorks. Each handles auth, extraction, and format translation, isolating the core from vendor-specific APIs.	System Admin	**Core**	**High**	US-1, US-2	★

Feature Table (3 of 3)

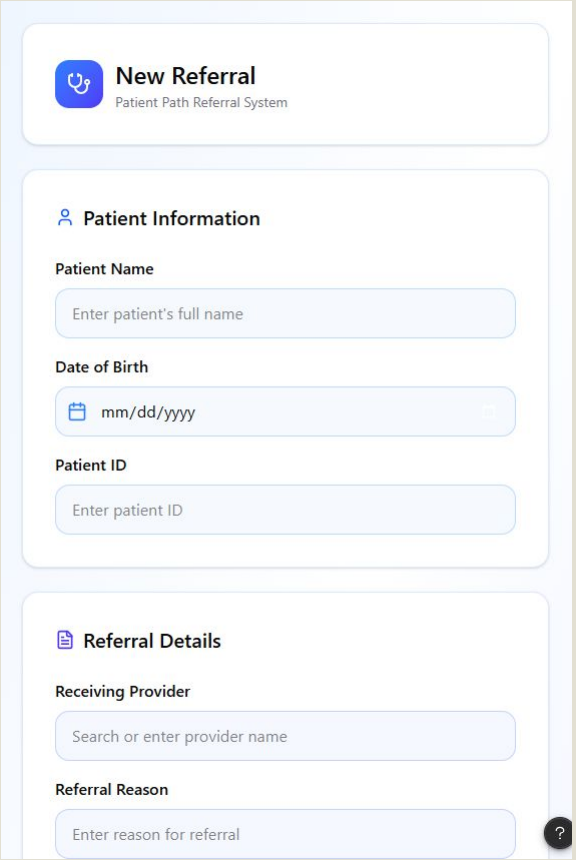
★ = Novel / Differentiating Feature

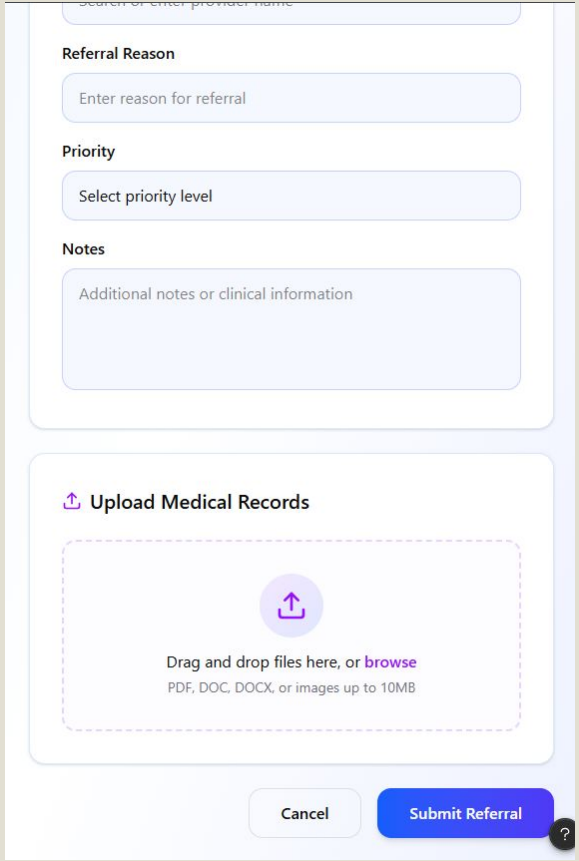
Feature	Description	User Roles	Priority	Complexity	Maps To	★
Interoperability (cont.)						
API Gateway	Central entry point for all client-server and system-to-system communication. Handles TLS termination, request routing, rate limiting, and auth token validation before forwarding to internal services.	System Admin	**Core**	**Medium**	US-1, US-2, US-11	
Performance & Reliability						
Cloud-Based Infrastructure	Horizontally scalable deployment on AWS, Azure, or GCP. Auto-scaling groups respond to load spikes; infrastructure defined as code (IaC) for reproducible deployments. Targets 99.9%+ uptime per SLA.	System Admin	**Core**	**Medium**	All	
Offline Sync Capability	Service Worker-based offline caching stores recently accessed records locally during network outages. Queued writes held in a local sync queue and reconciled with the server within 60 seconds of reconnection. Conflict resolution uses server-state as source of truth.	Provider	**Core**	**High**	US-5	★
High Availability Architecture	Active-passive failover with cloud provider SLA requiring 99.9%+ uptime. Database replication and load balancing ensure no single point of failure. Failover is automatic and transparent to all users.	All	**Core**	**High**	All	

User Interface Mockups : Provider Dashboard



User Interface Mockups : Referral Management





User Interface Mockups : Admin Console

Patient Path

Dashboard
Referrals
Users
Reports
Settings

Referral Management

Search referrals...

JD

Total Referrals	Pending	Approved	Completed
248	42	156	50
+12% from last month	Awaiting review	In progress	This month

Filters

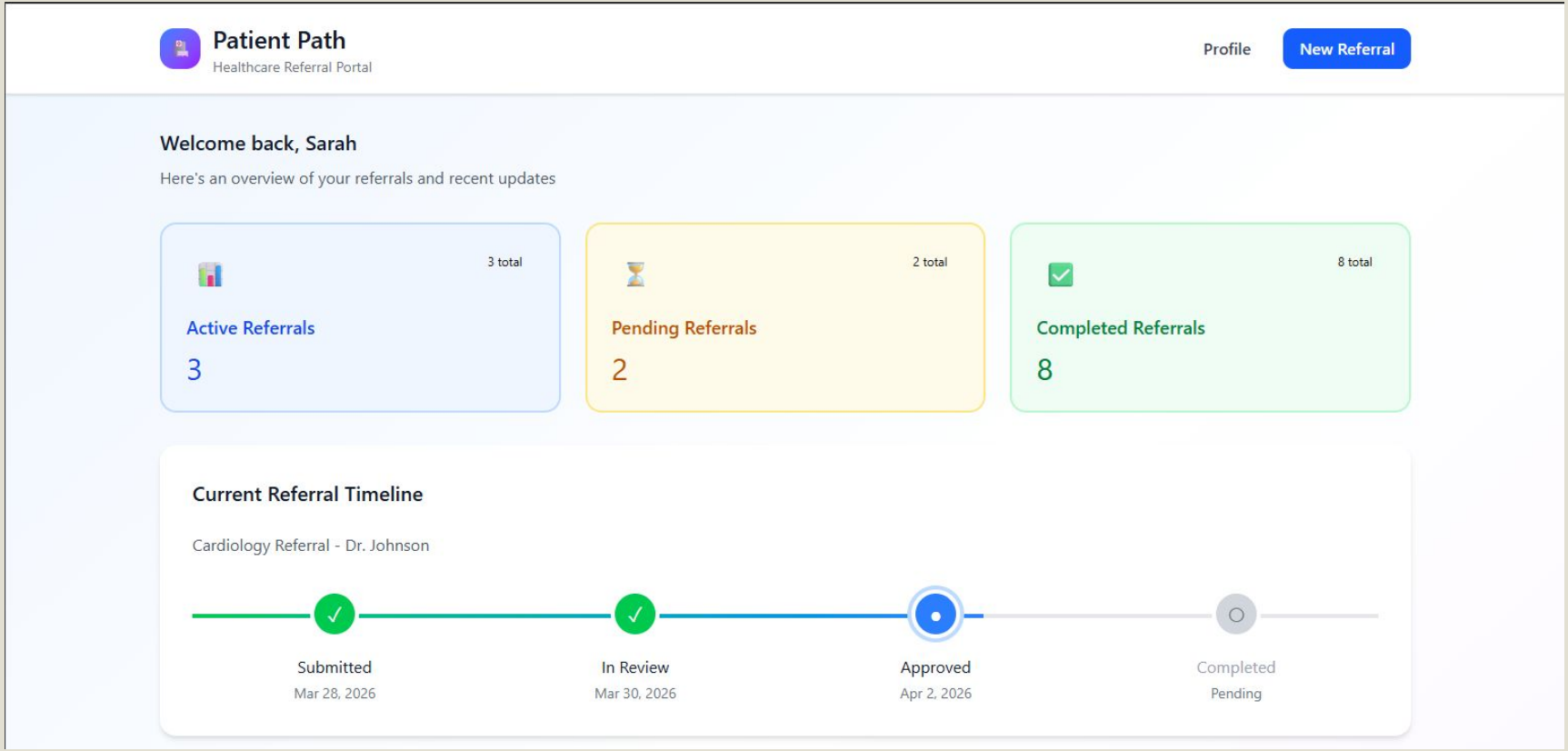
Status: All Statuses

Provider: All Providers

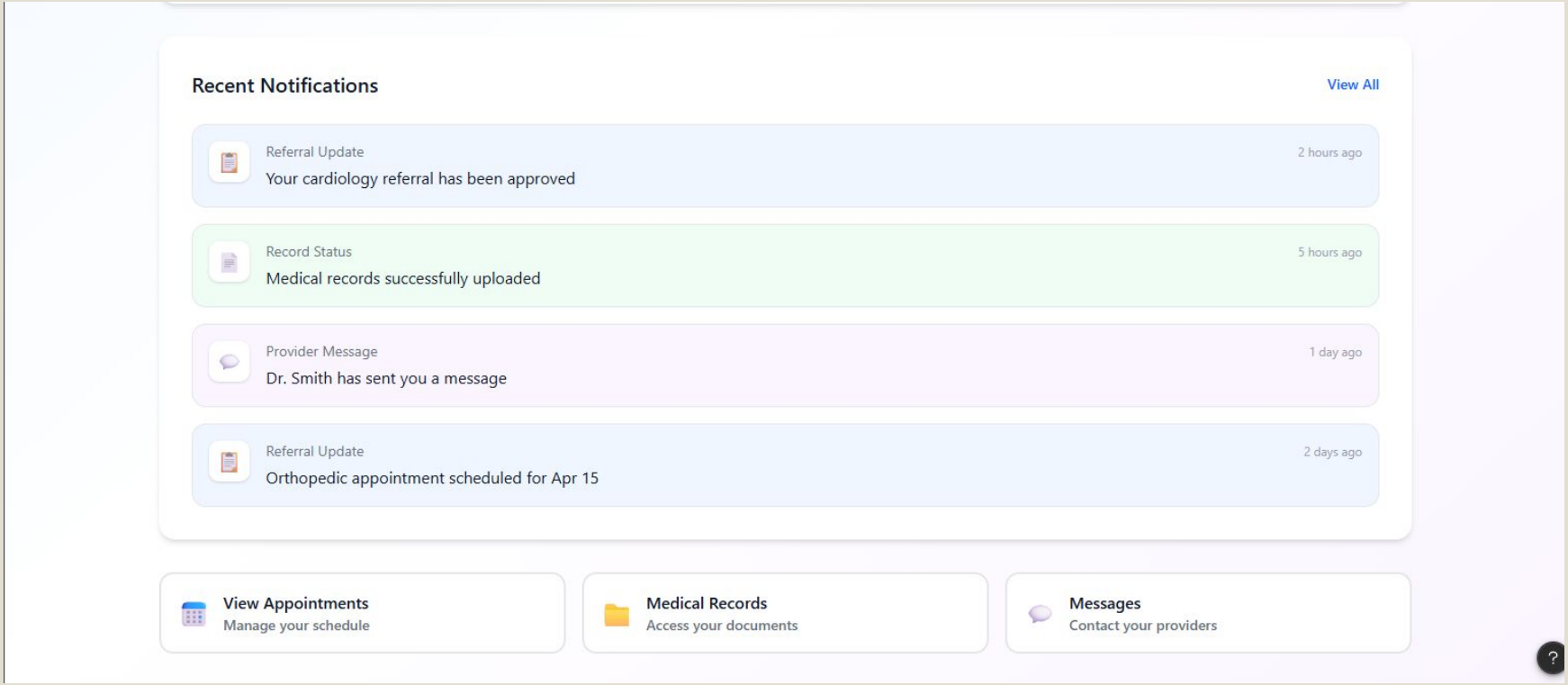
Date Range: mm/dd/yyyy

Referral ID	Patient Name	Sending Provider	Receiving Provider	Status	Last Updated	Actions
REF-2026-001	Sarah Johnson	Dr. Michael Chen	Dr. Emily Rodriguez	Pending	2026-04-05 09:30 AM	
REF-2026-002	James Williams	Dr. Lisa Park	Dr. Robert Kim	Approved	2026-04-05 08:15 AM	
REF-2026-003	Maria Garcia	Dr. David Lee	Dr. Jennifer Wong	Completed	2026-04-04 04:45 PM	
REF-2026-004	Robert Anderson	Dr. Michael Chen	Dr. Sarah Miller	Pending	2026-04-04 02:20 PM	
REF-2026-005	Linda Martinez	Dr. Emily Rodriguez	Dr. David Lee	Rejected	2026-04-04 11:30 AM	
REF-2026-006	Michael Brown	Dr. Robert Kim	Dr. Lisa Park	Approved	2026-04-03 03:15 PM	

User Interface Mockups : Patient Portal pt1



User Interface Mockups : Patient Portal pt2



Work Breakdown Structure

Work Breakdown Structure

PATIENT PATH SYSTEM

1. User Interface Design

1.1 Research & Planning
- Identify user roles
- Review user stories
- Define UI requirements

1.2 Wireframing
- Provider Dashboard
- Referral Form
- Admin Console
- Patient Portal

1.3 Mockup Design
- High-fidelity mockups
- Layout consistency
- Navigation design

1.4 UI Testing & Feedback
- Review with team
- Revise mockups
- Finalize UI screens

2. Algorithm Design

2.1 Referral Processing Logic
- Referral validation
- Status tracking
- Referral routing

2.2 Security & Access Control
- Role-based access
- Authentication
- Authorization checks

2.3 Data Validation
- File validation
- Input validation
- Error handling

2.4 Notification System
- Alert generation
- Status notifications
- Expiration alerts

3. Database Design / Schema

3.1 Identify Entities
- User
- Patient
- Referral
- Medical Record
- Consent / Org

3.2 Define Relationships
- User↔Org
- Patient↔Referral
- Referral↔MedRec
- Referral↔Consent

3.3 Tables & Attributes
- Primary keys
- Foreign keys
- Constraints

3.4 Schema Validation
- CRUD mapping
- Relationship validation

4. Data Management Approach

4.1 Storage Strategy
- Database selection
- Data structure planning

4.2 Data Security
- Encryption
- Secure file storage
- Access control

4.3 Data Access & Retrieval
- API endpoints
- Query optimization
- Data filtering

4.4 Data Maintenance
- Backup strategy
- Data cleanup
- Logging & auditing

Algorithm 1: ValidateAndEncrypt

ValidateAndEncrypt

User Stories: US-1, US-6

Inputs

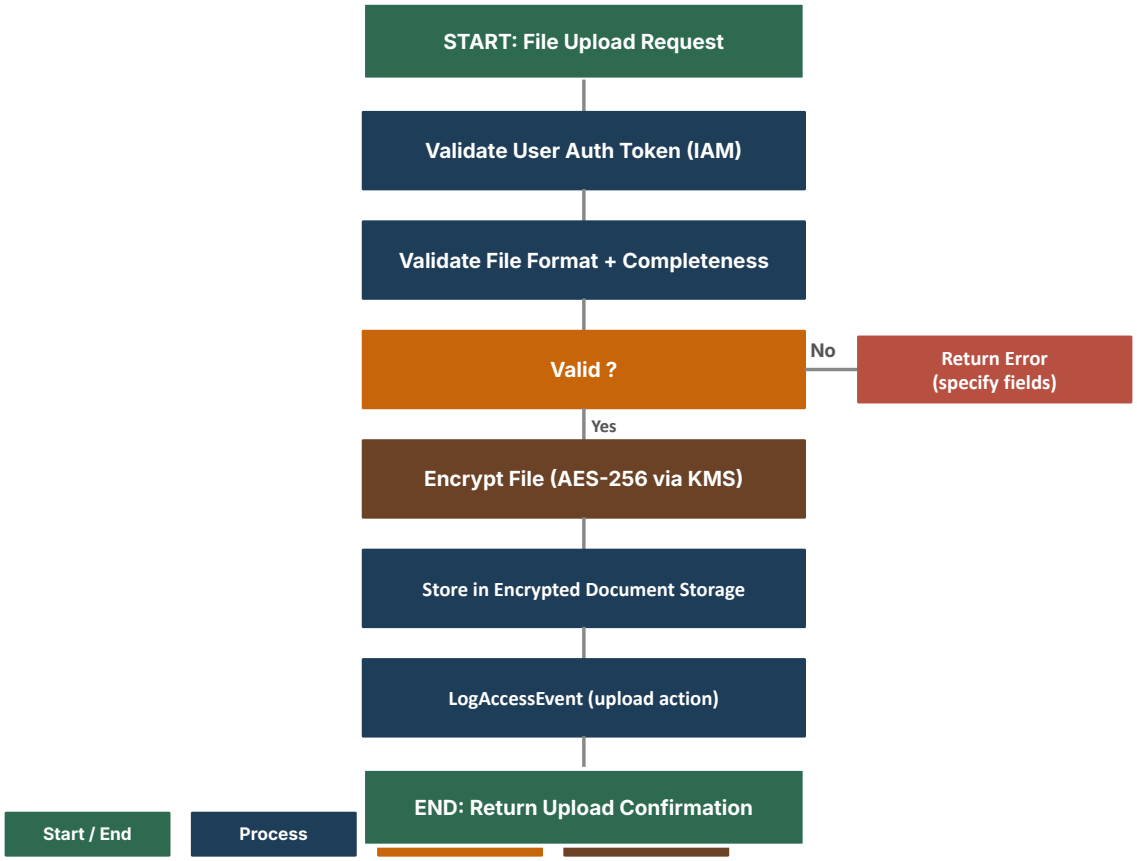
- File upload request
- User auth token
- Record metadata

Outputs

- Encrypted file in Document Storage
- Upload confirmation
- Audit log entry

External APIs

- AWS KMS / Azure Key Vault (AES-256 key management)



Algorithm 2: CreateReferral

CreateReferral

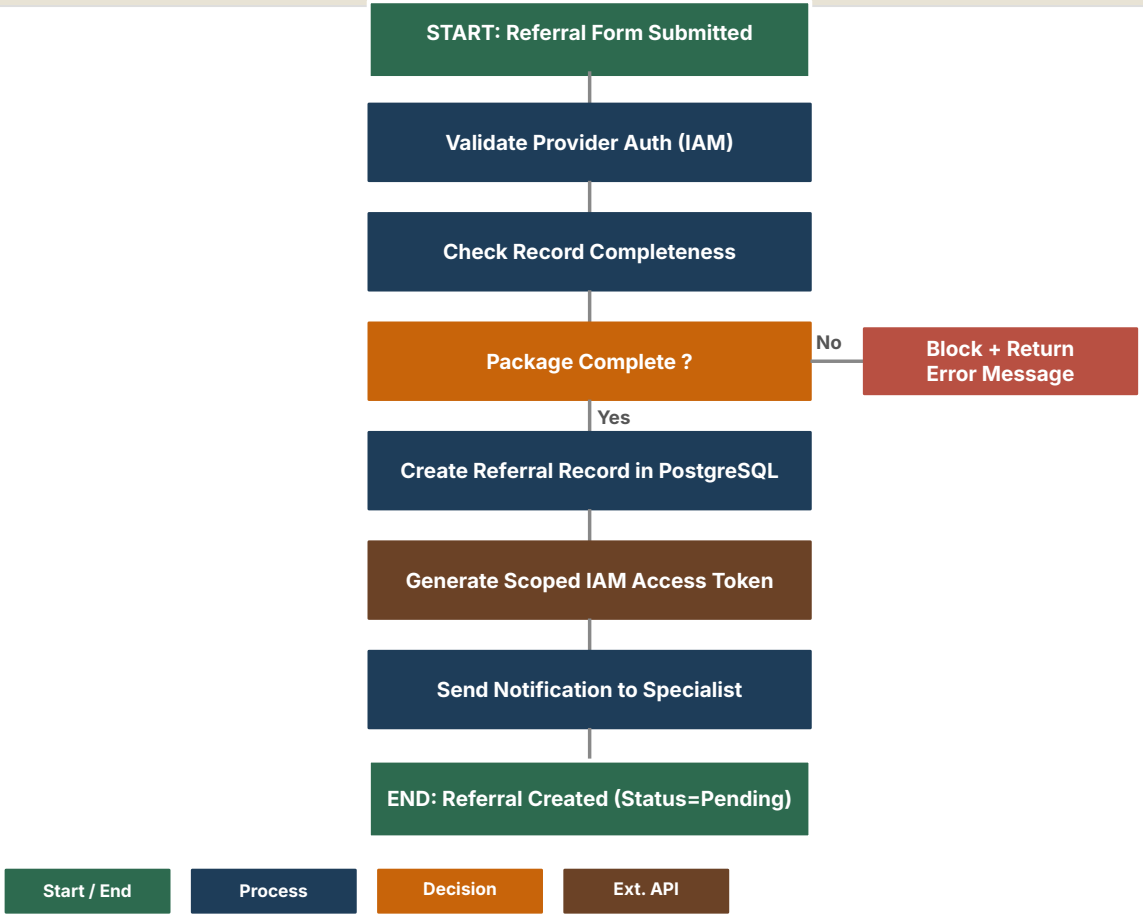
User Stories: US-2, US-3

Inputs

- Provider form submission
- Patient ID, specialist org, record IDs

Outputs

- Referral record in PostgreSQL
- Scoped IAM access token
- Notification to specialist



Algorithm 3: TrackReferralStatus

TrackReferralStatus

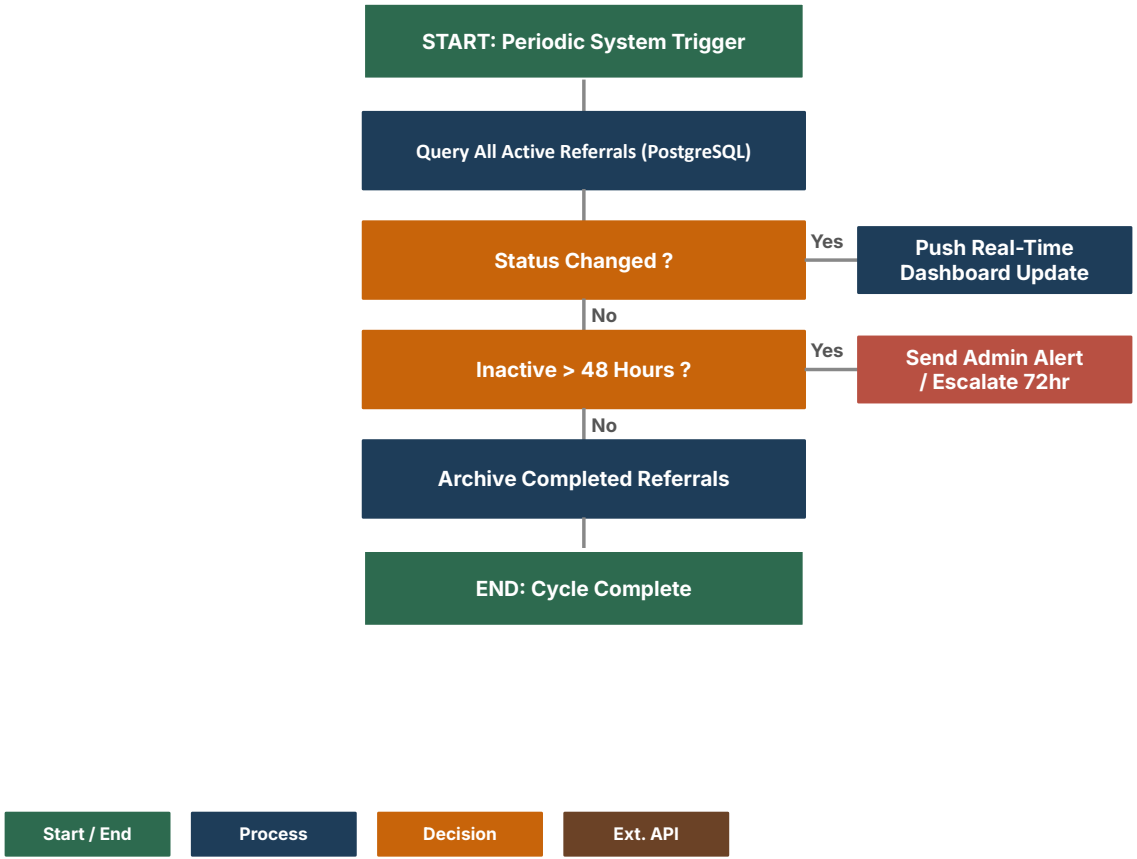
User Stories: US-3, US-8

Inputs

- Periodic system trigger
- Active referral records

Outputs

- Real-time dashboard updates
- Admin alerts (48hr)
- Supervisor escalations (72hr)



Algorithm 4: ManageConsent

ManageConsent

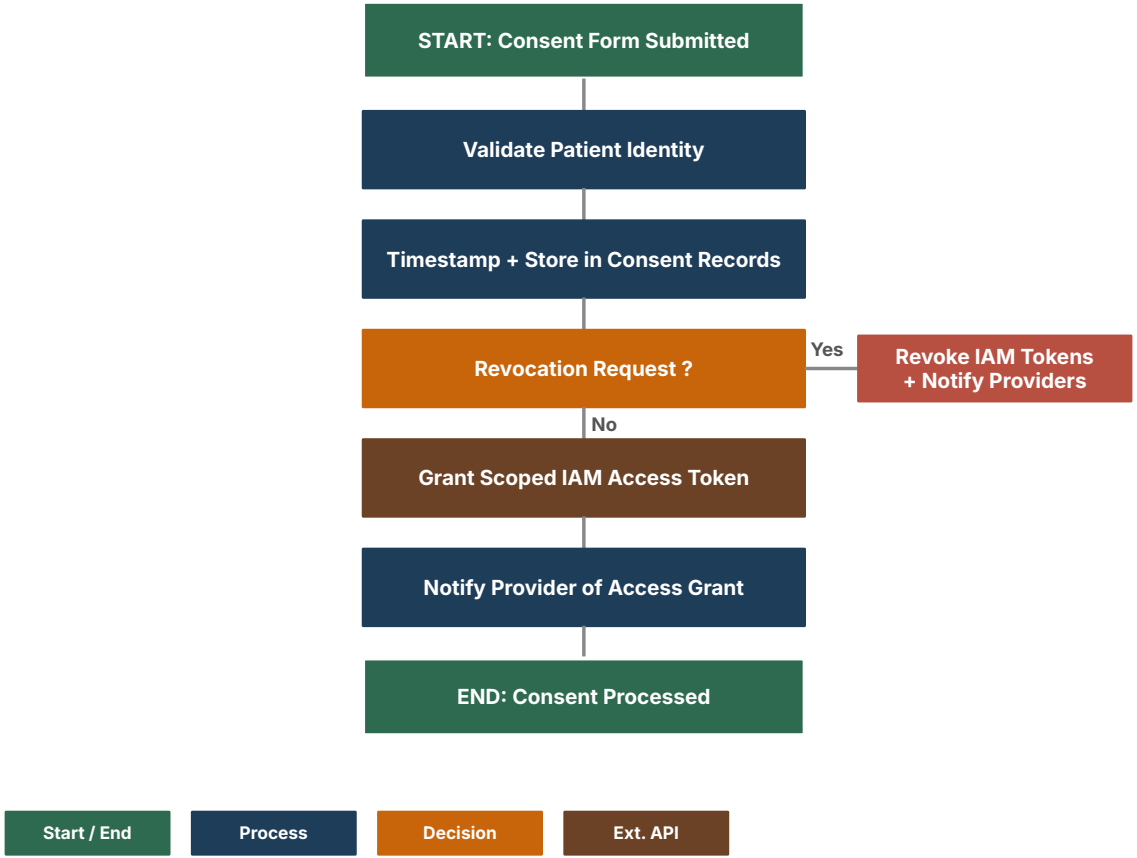
User Stories: US-9

Inputs

- Patient consent form submission
- Patient ID + provider org

Outputs

- Timestamped consent record
- IAM token grant / revocation
- Provider notification



Algorithm 5: LogAccessEvent

LogAccessEvent

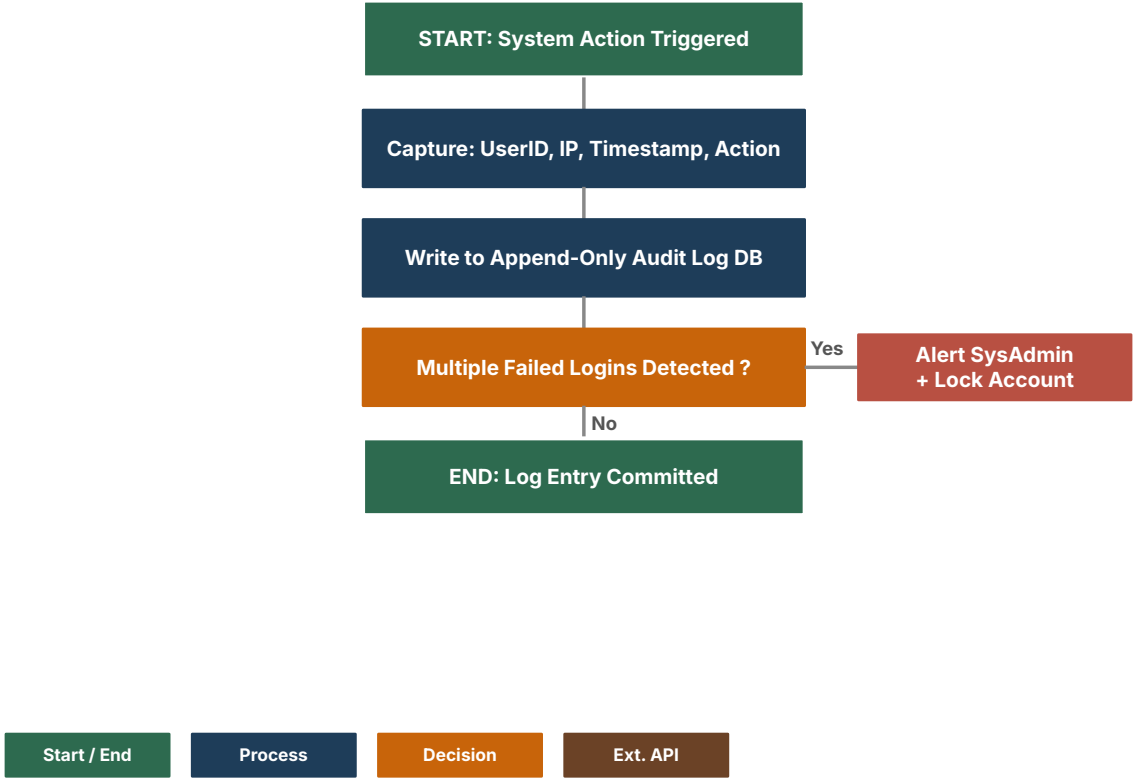
User Stories: US-7, US-11

Inputs

- Any system action (access, transfer, login)

Outputs

- Immutable audit log entry
- SysAdmin alert (if suspicious)



Algorithm 6: CacheRecords

CacheRecords

User Stories: US-5

Inputs

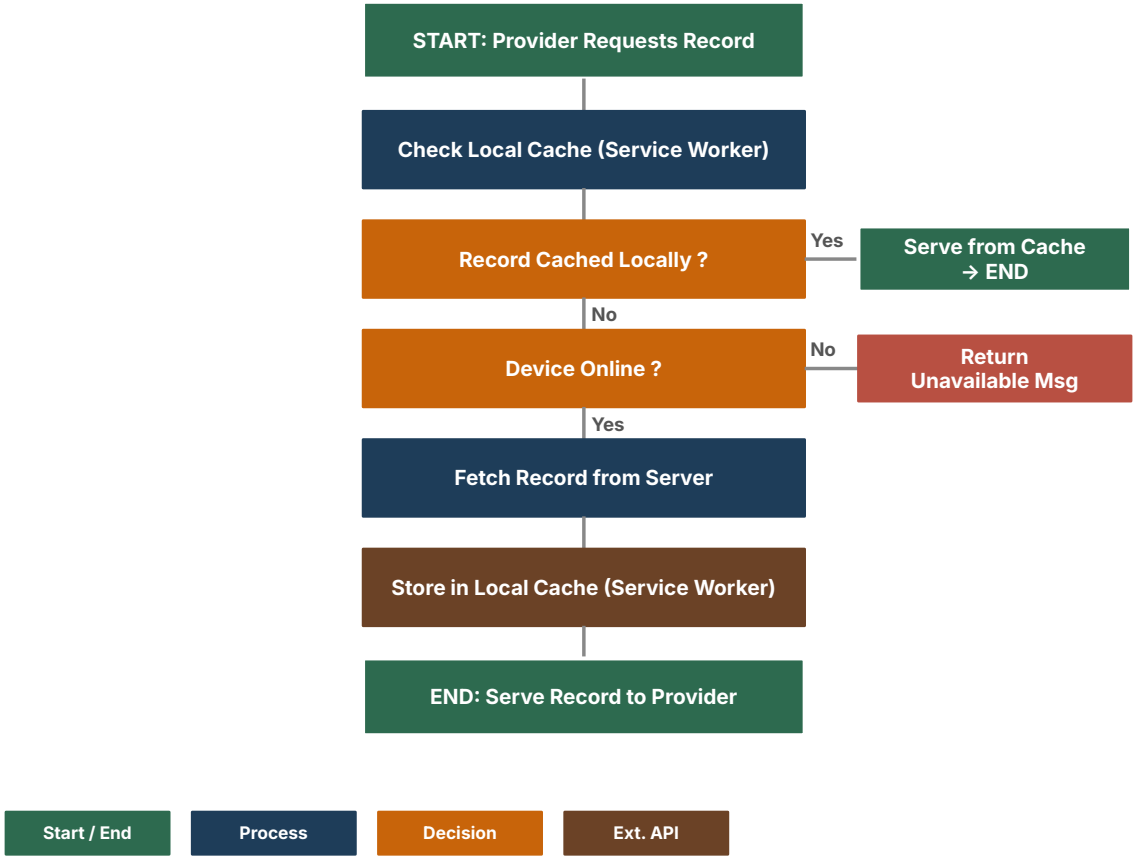
- Provider record access request

Outputs

- Record served to provider
- Local cache updated (if online)
- Unavailable message (if offline)

External APIs

- Service Worker (browser cache API)



Algorithm 7: SyncQueue

SyncQueue

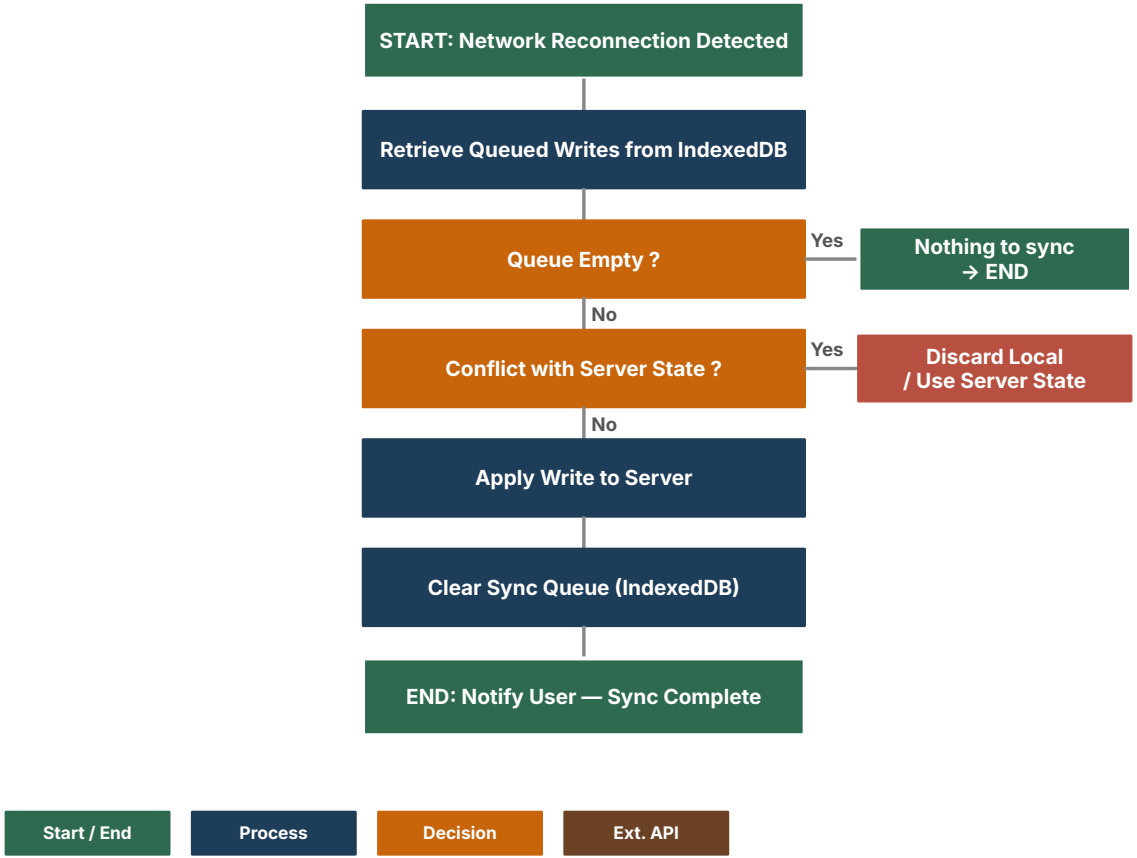
User Stories: US-5

Inputs

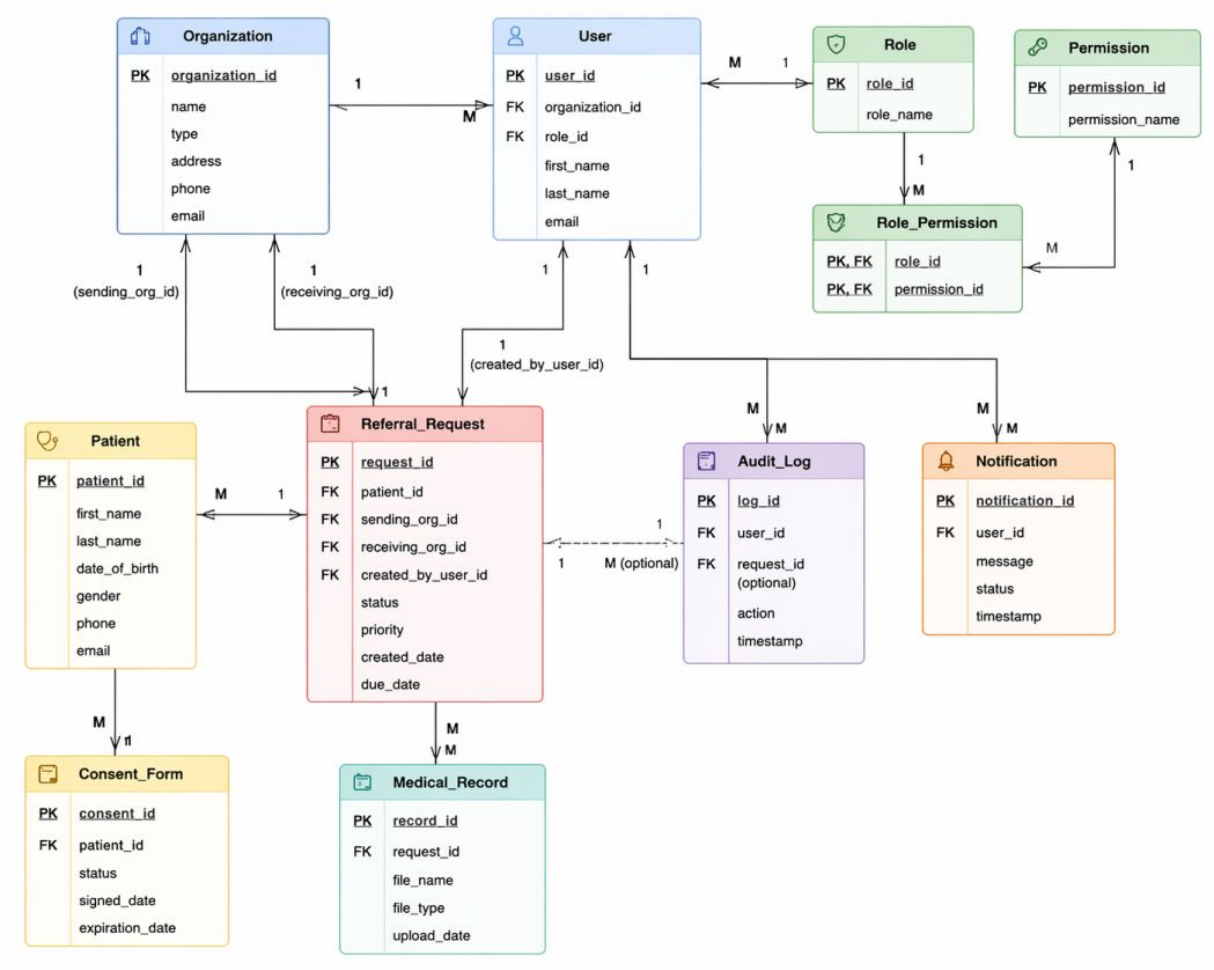
- Network reconnection event
- Local write queue (IndexedDB)

Outputs

- Writes applied to server
- Conflicts resolved (server wins)
- Sync confirmation to user



Database Schema



External Interface

AWS KMS / Azure Key Vault - Manages encryption keys for securing patient records (AES-256 Encryption).

Service Worker - Enables offline access, local caching, and sync of records when connection returns.

Identity Access Management (IAM) - Handles authentication, role-based access control, and secure access tokens.

Development Tools

Development Type	Front-End	Back-End
Language	TypeScript, HTML5, CSS	TypeScript
Framework	React	Node.js + Express.js
IDE	VS Code	VS Code
Version Control	GitHub	GitHub
CI/CD	GitHub Actions	GitHub Actions
Testing Framework	Jest	Jest
API	REST API	REST API
Database		PostgreSQL

Development Tools

HTML5 & CSS - Standard languages for web page structure and styling

TypeScript - Adds type checking to JavaScript which helps reduce mistakes in healthcare related workflows

VS Code - Easy to use, lightweight IDE that supports both front-end and back-end development

Node.js + Express.js - Used together to build the backend server and handles API requests, logic, and database communications

REST API - API architectural style that allows different software systems to communicate over the internet, it supports core actions like retrieving records, submitting referrals, updating statuses, and managing users

Development Tools

PostgreSQL - Relational database system that stores structured data like users, referrals, and records

Jest - Testing framework that automates unit, integration, and snapshot testing for TypeScript

GitHub - Most commonly used version control platform used to store code, track issues, and allows easy team collaboration through commits, branches, and pull requests

GitHub Actions - GitHub automation tool that runs project tasks whenever code is pushed to make sure the project is stable

React - Framework for building UI with reusable components

Sprint Breakdown

Sprint 0: Project setup

- Initialize GitHub repository and CI/CD pipeline.
- Set up project structure for **TypeScript** and **React**.
- Draft initial **PostgreSQL** schema for users, referrals, and records.
- Configure HIPAA security headers for web server.

Sprint 1: Identity & Access

- **UI:** User registration and MFA login screens using **React** components.
- **Logic: Node.js + Express.js** service for Role-Based Access Control (RBAC).
- **DB:** Create **PostgreSQL** tables for users and organizational data.

Sprint 2: Secure Record Exchange

- **UI: React** dashboard for instant document upload and retrieval.
- **Logic: Node.js** real-time transfer service using **REST API** architecture.
- **DB:** Securely store medical records and metadata in **PostgreSQL**.

Sprint 3: Interoperability

- **UI: REST API** connection status and data mapping dashboard.
- **Logic: Node.js** FHIR R4 / HL7 Integration Service for EHR compatibility
- **Algorithm: TypeScript** FHIRTranslator module for data normalization.

Sprint Breakdown

Sprint 4: Automated Referrals

- **UI: React** interface for initiating referrals with one-click record selection.
- **Logic: TypeScript** ReferralValidator module for automated data checks
- **Workflow:** Real-time status tracking via **Node.js** and **REST API**.

Sprint 5: Offline Resilience

- **UI:** Offline mode indicators and cached **React** dashboard view.
- **Logic: TypeScript** SyncService for background synchronization.
- **DB:** Utilize browser storage for local caching during connectivity drops.

Sprint 6: Patient Portal

- **UI: React** portal for patients to view records and manage consent.
- **Logic: REST API** module for secure record release requests.
- **DB: PostgreSQL** tables for consent records and access keys.

Sprint 7: Polish & Finalize

- **Logic:** Finalize immutable audit trail logging in **PostgreSQL**.
- **Testing:** Automated unit and integration testing using the **Jest** framework.
- **UI:** Final **React** dashboard refinement and navigation polishing.

Conclusion

PatientPath modernizes healthcare communication by replacing outdated fax-based processes with a secure, efficient digital platform. By improving access to medical records, reducing administrative burden, and strengthening data security, PatientPath helps providers deliver
faster, safer, and more coordinated care. In a healthcare system where time and accuracy matter, PatientPath provides a necessary step forward.

References

Office of the National Coordinator for Health Information Technology. (2023). *Interoperable exchange of patient health information among U.S. hospitals: 2023 data brief*. U.S. Department of Health & Human Services. https://www.healthit.gov/data/data-briefs/interoperable-exchange-patient-health-information-among-us-hospitals-2023

U.S. Department of Health & Human Services. (2013). *Summary of the HIPAA security rule*. https://www.hhs.gov/hipaa/for-professionals/security/laws-regulations/index.html

HL7 International. (2023). *FHIR overview*. https://www.hl7.org/fhir/overview.html

American Hospital Association. (2023). *AHA annual survey IT supplement*. https://www.aha.org

Vyshyvaniuk, K. (2026). *Top 7 EHR implementation challenges and how to overcome them*. https://kitrum.com/blog/ehr-implementation-challenges/

Kruse, C. S.Basil, N. N., Ambe, S., Ekhator, C., & Fonkem, E. (2022). *Health Records Database and inherent security concerns: A review of the literature*. https://pmc.ncbi.nlm.nih.gov/articles/PMC9647912/

Kristof, C., Jones, B., Mitchell, E., & Martinez, A. (2016). *Barriers to electronic health record adoption: A systematic literature review*. https://pmc.ncbi.nlm.nih.gov/articles/PMC5054043/

Clarke, M., & Martin, K. (2024). *Managing cybersecurity risk in healthcare settings*. https://pmc.ncbi.nlm.nih.gov/articles/PMC10725101/

Ajami, S., & Bagheri-Tadi, T. (2013). *Barriers for adopting electronic health records (ehrs) by physicians.* https://pmc.ncbi.nlm.nih.gov/articles/PMC3766548/

Glossary

PCP - Primary Care Provider
EHR - Electronic Health Records
HIPAA - Health Insurance Portability and Accountability Act
FHIR - Fast Healthcare Interoperability Resources
RBAC - Role-Based Access Control
HL7 - Health Level Seven